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Assistant Secretary for Technology Policy (ASTP)

Office of the National Coordinator for Health Information Technology (ONC)

Health Technology Ecosystem

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Cover Page

Centers for Medicare & Medicaid Services (CMS)
Office of Burden Reduction & Health Informatics
7500 Security Boulevard
Baltimore, MD 21244

RE: Response to CMS RFI on Advancing Interoperability and Improving Prior Authorization Processes (CMS-0057-F Follow-On Questions)

Dear CMS Review Team,

On behalf of RELI Group, Inc., we appreciate the opportunity to provide expert feedback in response to CMS' Request for Information on advancing healthcare interoperability. Our team brings extensive experience supporting CMS data modernization, interoperability strategy, and regulatory alignment initiatives. The enclosed responses address the following four questions:

- TD-3: Challenges, benefits, and strategic implications of digital identity credential adoption
- TD-12: CMS endorsement of non-CMS data sources and networks
- TD-13: Opportunities, obstacles, and tradeoffs in Application Programming Interface (API)-based full Electronic Health Information (EHI) access
- TD-17: CMS/ONC roles in ensuring viability of healthcare data sharing networks

We commend CMS for taking a policy-forward approach in this RFI. In our responses, we have emphasized actionable recommendations that reinforce federal leadership while enabling the market to scale sustainable, standards-driven solutions. Each response reflects our ongoing work with CMS programs, private-sector stakeholders, and national networks dedicated to patient-centered, secure, and equitable data exchange.

We welcome any further dialogue or clarification CMS may seek and remain committed to supporting this important work.

Sincerely,

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A. COMPANY DESCRIPTION

For more than 10 years, RELI Group Inc. has made the mission of the Centers for Medicare & Medicaid Services (CMS) our priority, specifically delivering impactful solutions and services that provide critical information to beneficiaries and providers. During this time, we have grown from a small business to a mid-size company with industry certifications that attest to our capabilities, including Capability Maturity Model Integration (CMMI) Maturity Level 3 (CMMI L3 DEV, SVCS, DATA, SEC, SPM) and ISO 9001:2015, 20000-1:2018 and 27001:2022. Our longstanding support of CMS runs deep, with experience working alongside numerous centers and offices such as the Office of Financial Management (OFM), Office of Information Technology (OIT), Center for Program Integrity (CPI), Office of Communications (OC), Center for Clinical Standards and Quality (CCSQ), Center for Medicare (CM), Center for Medicare and Medicaid Innovation (CMMI), and Center for Consumer Information and Insurance Oversight (CCIIO).

Understanding the Requirement. We recognize the need for CMS to cultivate a market that eliminates fraud, waste, and abuse while ensuring secure communications that leverage CMS-managed agile approaches. The goal is to improve collaboration and simplify user experience by moving from a silo-based model to a fully interoperable patient data ecosystem — while offering seamless, protected, fine-grain data across systems that foster patient trust.

RELI is uniquely positioned with deep experience currently working with CMS offices to improve data access in the digital health and data interoperability space to include:

- National Plan & Provider Enumeration System (NPPES) - CMS Center for Program Integrity (CPI)
- Multidimensional Information Data Analytics System (MIDAS) - CMS Center for Consumer Information and Insurance Oversight (CCIIO)
- Medicare Exclusion Database (MED) - CMS CPI
- Data Exchange (DEX) - CMS CPI

CMS needs a contractor with a deep understanding of public-facing needs via harvesting deep health IT experience at CMS, agile processes and methodologies that enhance user experience, and program strategies that optimize strong governance and federated identities within a trusted health ecosystem. With highly qualified subject matter experts, RELI is uniquely positioned to support this effort.

2. DIGITAL IDENTITY

TD 3. Regarding digital identity implementation --

TD-3(a): What are the challenges and benefits?

Challenges

Equity and access must be proactively addressed via robust authorization controls and consent management to effectively achieve patient/caregiver efficiencies. Digital identity systems must accommodate individuals with limited connectivity, low digital literacy, language barriers, or lack of formal documentation. Absent inclusive design, these systems risk deepening existing disparities.

Privacy and trust concerns must also be mitigated. Patients may resist identity systems perceived as intrusive, particularly those using biometrics or centralized databases. Transparent governance, patient-controlled sharing, and strong privacy protections are essential.

On the technical side, legacy systems often **lack native support** for OAuth or OpenID Connect, requiring integration layers or modernization efforts. Small and under-resourced providers may lack the capital or expertise for implementation, further widening the digital divide.

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Governance challenges emerge when digital identity spans organizational boundaries. Credential Service Providers (CSPs) may apply varying identity proofing standards, and healthcare providers relying on those credentials may lack visibility into their rigor. This can create ambiguity in determining liability for authentication errors or unauthorized access. Additionally, urgent care scenarios often require policy exceptions, further complicating consistent enforcement.

Finally, **regulatory alignment** is lacking. HIPAA, 42 CFR Part 2, and state laws offer inconsistent guidance on identity verification. Without federal standards or incentives, adoption remains fragmented and uneven.

Benefits

From a patient perspective, digital identity solutions **eliminate the fractured access landscape** currently plaguing the system and eroding patient trust. Rather than managing disparate credentials across providers, pharmacies, insurers, and health applications, patients could use a single, portable digital identity throughout their care journey. This enables consistent authentication, delegated caregiver access, and granular consent management. These features are especially **impactful for complex patients or those with family-based care coordination**.

Clinically, accurate patient identification improves safety by reducing record mix-ups and enhances care quality through access to complete histories. Identity verification during care transitions **ensures accurate medication reconciliation and timely treatment**. These capabilities are foundational to safe, coordinated care and foster maximum patient care impact.

Operationally, digital identity **reduces the administrative burden** of account recovery, duplicate record correction, and manual verification. Workflow efficiency increases as patients arrive with pre-verified identities. Accurate identity also improves revenue cycle performance by reducing registration and claim processing errors.

From a cybersecurity standpoint, high-assurance credentials reduce vulnerabilities to phishing, credential stuffing, and account takeovers. They **enable better audit trails**, fraud detection, and risk governance — key for HIPAA compliance and protecting sensitive data.

At the system level, consistent identity **supports longitudinal patient tracking** for value-based care, enhances public health surveillance, and improves data quality for research. Digital identity is a linchpin of next-generation interoperability.

> ***TD-3(b): How would requiring digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs) impact cybersecurity and data exchange?***

Requiring standardized credentials, such as those issued by CLEAR, Login.gov, or ID.me, would elevate healthcare's cybersecurity and interoperability maturity.

Cybersecurity Impacts

AAL2-compliant multi-factor authentication would create a uniform identity security baseline, reducing phishing, synthetic identity fraud, and unauthorized access. CSPs typically maintain stronger security controls, centralized monitoring, and more rigorous patch management than many healthcare organizations or IT system operators can implement independently. Standardization also **enhances anomaly detection and incident response**.

Audit trails tied to verified identities improve attribution and forensic analysis. Federated identity models support **cross-organizational threat correlation**. Consistent enforcement of access controls across systems strengthens governance, compliance, and risk posture.

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Data Exchange Impacts

The United States lacks a national patient identifier, relying instead on probabilistic matching methods that are error-prone and contribute to data fragmentation. Federated identity frameworks, anchored in verified digital credentials, offer a scalable alternative. By enabling cross-organizational identity resolution, they **improve the accuracy of patient matching** and reduce redundant or conflicting records.

Trusted credentials eliminate the need for each organization to independently verify identity, streamlining access to data and services. Consent artifacts and access permissions become **more portable, enforceable, and auditable across systems**.

Directory services and patient lookup mechanisms benefit from higher-quality input, improving both confidence and performance. Smaller providers, which often lack the resources to build identity infrastructure, gain access to secure exchange networks by relying on federated credentials. Under a common identity framework, provenance tracking, access auditing, and secure data routing are significantly enhanced, supporting both interoperability and regulatory compliance.

Risks and Implementation Considerations

CSPs become high-value targets. A compromise could impact many healthcare entities, so risk must be mitigated through isolation, redundancy, and breach protocols. Real-time verification adds performance overhead. System migrations and interoperability across CSPs require coordination.

Despite these challenges, requiring standardized credentials would greatly enhance security and streamline trusted exchange.

TD-3(c): What impact would mandatory use of the OpenID Connect identity protocol have?

Mandating OpenID Connect (OIDC) would unify authentication infrastructure across healthcare.

Operational Benefits

OIDC would standardize workflows, reduce integration costs, and simplify onboarding. Health IT vendors could align with a consistent protocol, accelerating adoption. Patient experiences would improve with familiar login flows and intuitive delegation options.

Security and Risk Management

OIDC's token-based authentication supports granular authorization, session control, and token revocation. It aligns with zero-trust architecture and supports least-privilege enforcement. Threat models become more consistent, and security testing benefits from established tools.

Governance and Implementation

Transition requires migration support for legacy systems. CMS should implement phased timelines, reference and sample architectures, testing tools, and certification programs. Governance must define responsibilities, handle disputes, and enforce compliance.

Stakeholder Engagement

Successful adoption hinges on inclusive planning. Patient advocates, identity federations, developers, and provider networks should all have input. Collaboration with standards bodies will support healthcare-specific protocol extensions.

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Conclusion

Mandatory OIDC adoption would modernize and unify the healthcare identity architecture. With proper support, it would enable secure, scalable, and consistent identity management across the ecosystem.

4. DATA EXCHANGE

TD 12. Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

Summary

CMS should establish a formal, standards-based endorsement framework for non-CMS data sources and networks. By doing so, CMS can accelerate the development of longitudinal patient records that span clinical and administrative domains — a foundational requirement for effective value-based care, public health response, and program integrity oversight. Endorsement is not simply a signal of trust; it is a mechanism to align market participants with federal priorities such as CMS-0057-F, Trusted Exchange Framework and Common Agreement (TEFCA), and the broader national interoperability agenda.

Strategic Rationale

Fragmented health data ecosystems continue to limit progress on care coordination, data-driven payment innovation, error detection/reconciliation, and the consistent delivery of high-quality services across populations. An endorsement framework helps overcome these limitations by encouraging alignment with national standards, reducing variation in data access, and enabling smaller, under-resourced organizations to participate on equal footing that fosters an inclusive ecosystem.

CMS should focus on endorsing networks and platforms that enable:

- **USCDI (United States Core Data for Interoperability) and FHIR-based exchange (Fast Healthcare Interoperability Resources)**, laying the foundation for shared semantics and standardized payloads.
- **Full EHI access**, enabling providers and payers to construct rich, longitudinal views of patient history — including problem lists, labs, medications, and encounter data.
- **All-Payer Claims Database (APCD) integration**, enhancing CMS' ability to contextualize Medicare and Medicaid data within broader population health and utilization patterns.

Together, these components allow CMS to shift from episodic, siloed data environments toward dynamic, longitudinal records that support both real-time operations and retrospective analytics.

Expanded Benefits of Endorsement

- **Accelerates Longitudinal Data Creation**
  Endorsed networks that support USCDI, TEFCA routing, and payer data aggregation become critical infrastructure for assembling comprehensive patient records. CMS leadership can bridge the technical and policy divide between claims and clinical data silos.
- **Stimulates All-Payer Ecosystem Participation**
  By endorsing networks that integrate with state APCDs and commercial payers, CMS increases

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the analytic fidelity of multi-payer models and supports more equitable benchmarking across populations.

- **Reduces Redundancy and Complexity for Providers**
  A CMS-backed list of endorsed exchange partners reduces the burden on providers to perform bespoke integrations, especially for those participating in multiple federal and state programs.
- **Enhances Program Integrity and Surveillance**
  Federated access to complete patient records improves the precision of fraud, waste, and abuse detection while also enabling near real-time monitoring of health outcomes across programs.
- **Catalyzes Investment and Innovation**
  Clear CMS endorsement criteria offer the private sector a pathway for platform validation, derisking innovation around digital identity, record location services, and bidirectional data exchange.

Recommended Endorsement Criteria

1. **Technical Standards Compliance**
   - Full alignment with FHIR R4, USCDI v3+, OAuth 2.0, OpenID Connect
   - Ability to support bidirectional queries and responses using SMART on FHIR applications
   - Proven capacity for identity resolution and patient-matching algorithms
2. **Data Stewardship and Quality**
   - Conformance testing frameworks and performance dashboards
   - Structured logging for provenance and auditability
   - Support for longitudinal record construction with normalization logic
3. **Governance and Trust Frameworks**
   - Alignment with TEFCA principles (Common Agreement, QHIN connectivity)
   - Certification under programs such as HITRUST, NIST CSF, or SOC 2
   - Transparent data use policies, consent management, and appeal pathways
4. **Market Penetration and Network Effects**
   - Demonstrated support for national-scale use cases (e.g., payer-to-payer, prior auth, duals)
   - Adoption by multi-state providers, MA plans, Medicaid MCOs, and public health agencies
5. **Support for Public Objectives**
   - Ability to serve underserved populations and address geographic and population-based data gaps
   - Proven impact on reducing administrative burden or improving care coordination
   - Compatibility with CMS programs such as the Innovation Center models or the Quality Payment Program
6. **Operational Sustainability**
   - Business continuity planning, governance board structure, and funding transparency
   - Helpdesk and onboarding support for both technical and policy stakeholders

Implementation Approach

CMS should adopt a tiered endorsement model that reflects increasing levels of operational maturity, scalability, and demonstrated data quality to include:

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- **Provisional Endorsement**
  For networks meeting baseline interoperability and security criteria, including adherence to FHIR and USCDI standards, identity proofing protocols, and minimum data quality thresholds (e.g., completeness, timeliness, and conformance metrics).
- **Full Endorsement**
  For platforms that demonstrate operational scale, sustainability, and compliance with advanced performance metrics, including measurable data quality benchmarks such as validated record linkage, low error rates, and support for longitudinal record assembly.
- **Strategic Endorsement**
  For high-impact networks delivering systemic value, such as real-time USCDI ingestion for dual-eligible populations, integration across All-Payer Claims Databases (APCDs), and demonstrated ability to support comprehensive, high-fidelity patient records used in care delivery, analytics, or payment reform initiatives.

Each endorsement level should be reassessed every 24–36 months with public comment periods and transparent revocation pathways to maintain trust and flexibility.

Conclusion

The CMS endorsement of non-agency data networks is not merely symbolic, it is catalytic. With the emergence of payer-to-payer exchange, TEFCA connectivity, and full EHI access mandates, the opportunity to unify fragmented infrastructure into a learning health system has never been greater. An endorsement framework that emphasizes standards, trust, and shared value accelerates the shift to real-time, comprehensive patient data fueling care transformation — advancing health outcomes for all and preserving program integrity at scale.

TD 13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

New Opportunities and Advancements from Full EHI API Access

The transition from limited, siloed access to comprehensive, standards-based API access to a patient's full EHI represents a foundational leap in healthcare interoperability. While current standards such as USCDI lay the groundwork, full EHI access unlocks a broader data fabric that can meaningfully reshape care delivery, patient empowerment, and healthcare innovation. Core elements include:

1. Longitudinal Care Planning
Access to a patient's complete EHI allows providers and care coordination systems to understand the full arc of a patient's medical history, providing a history across conditions, providers, settings, and time. For patients with complex, chronic, or rare conditions, this longitudinal context is often essential to appropriate decision-making. Care gaps, subtle contraindications, and care redundancies become easier to identify when the full patient narrative is visible through API-connected platforms.

2. AI-Enabled Clinical Decision Support and Risk Stratification
With full EHI access, modern data science and AI tools can analyze unstructured data such as clinical notes, discharge summaries, and imaging narratives to surface insights that go beyond structured fields. These tools can flag early signs of deterioration, identify diagnostic inconsistencies, or recommend personalized therapies based on a patient's history, capabilities not feasible with a USCDI-only subset.

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3. Precision Public Health and Population Risk Management
De-identified, complete EHI datasets accessed through APIs can support next-generation public health surveillance and targeted population interventions. For example, complete medication histories and behavioral health narratives can help predict opioid overdose risk or detect emergent chronic conditions at the community level. This enables payers and regulators to direct interventions based on fine-grained, real-world data.

4. Research and Real-World Evidence Generation
Full EHI access with patient consent supports scalable, privacy-preserving medical research. Investigators can incorporate rich, real-world data. This includes lab trends, care team interactions, and historical context, that would otherwise be inaccessible. This accelerates time-to-evidence in comparative effectiveness studies and informs policy and coverage decisions with greater precision.

5. Patient Empowerment and Shared Decision-Making
True patient-directed access to full EHI enables individuals to manage their care proactively, share records across providers, and serve as the orchestrators of their own health data. Tools that integrate with patient-authorized APIs can consolidate fragmented records into personal health applications, eliminating the burden of tracking histories manually or requesting PDFs from disparate portals.

TD-13(a): What are the primary obstacles to this?

The technical and policy shifts required to realize full EHI access are significant. While the promise is clear, the following obstacles must be carefully managed:

1. Technical Barriers

- **Legacy Infrastructure**: Many EHR systems were not designed for full EHI export via API, especially when dealing with large volumes of historical, unstructured, or scanned data.
- **Unstructured Data Complexity**: Notes, PDFs, and imaging metadata are inconsistently coded and difficult to normalize across systems.
- **Performance and Scalability**: Delivering complete records on demand introduces bandwidth and latency concerns, especially under concurrent use.

2. Privacy and Consent Complexity

- **Granular Consent Requirements**: Full EHI often contains sensitive information (e.g., behavioral health, reproductive care, social risk factors). Current models struggle to support patient-friendly consent at the level of individual data elements or contexts.
- **Auditability and Risk Mitigation**: Increased access must be met with robust audit trails and user identity assurance to prevent misuse or unauthorized sharing.

3. Operational and Workflow Disruption

- **Provider Adoption Challenges**: Clinicians and administrators may be hesitant to release full EHI to external parties without confidence in downstream handling, provenance, and liability coverage.
- **Organizational Readiness**: Upgrading infrastructure, training staff, and validating data mapping across full EHI elements represent a significant burden, especially for small or under-resourced organizations.

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4. Data Quality and Governance Concerns

- **Variable Quality of Legacy Data**: Historical records may include inaccuracies, unverified patient-reported items, or incomplete clinical context.
- **Provenance and Lifecycle Management**: As EHI spans years or decades, determining the custodial source, context of creation, and expiration rules for every data element becomes more complex.

5. Regulatory Ambiguity

- **Overlap with Information Blocking:** While the 21st Century Cures Act mandates broad availability of EHI, definitions, enforcement boundaries, and exceptions, such as those related to patient harm or privacy, are still evolving. This creates uncertainty for implementers balancing access with legal risk.
- **Interplay with Other Rules:** 42 CFR Part 2, HIPAA, and state-level privacy laws may impose stricter protections on specific data elements, particularly behavioral health and substance use disorder (SUD) treatment records, than federal interoperability policy assumes. These protections often include redisclosure restrictions and detailed consent requirements that challenge seamless data exchange.
- **Segmentation and Consent Challenges:** To comply with regulations like Part 2, health IT systems must support the ability to segment sensitive data (e.g., SUD, reproductive health, behavioral health), manage consent directives, and enforce redisclosure limitations. However, most FHIR-based interoperability implementations today do not offer native support for such functionality. This leads to a compliance gap and discourages sharing, even when it is technically permissible.

Legal and Operational Misalignment

Interoperability policies often assume uniform trust and disclosure frameworks across all health data. In practice, variable obligations based on data type, origin, or jurisdiction create tension between federal interoperability objectives and on-the-ground legal compliance.

> ***TD-13(b): What are the primary tradeoffs between USCDI and full EHI, especially given more flexible data processing capabilities today?***

The distinction between USCDI and full EHI reflects a deeper strategic tension between standardization and completeness. The following table outlines our understanding of key tradeoffs.

Dimension	USCDI	Full EHI
Implementation Scope	Defined set of structured fields	Entire designated record set, including unstructured
Data Quality Assurance	Easier validation and conformance testing	Variable quality; harder to audit
Interoperability	High across vendors due to common profiles	Lower; dependent on local implementation

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Privacy Management	Easier to support consent and segmentation	Harder to control sensitive data exposure
Clinical Richness	Good for baseline interoperability	Essential for precision medicine and care planning
Cost and Complexity	Lower upfront cost, higher limitations	Higher investment, greater long-term value
Regulatory Alignment	Clearer pathways under ONC certification	Requires additional policy scaffolding

Strategic Implication

USCDI should serve as a policy anchor and implementation starting point. It defines the floor, not the ceiling, for interoperability. CMS should encourage phased expansion toward full EHI access through incentivized pilots, prioritized use cases (e.g., care transitions, value-based payment models), and ongoing alignment with TEFCA.

Modern data processing (e.g., AI/NLP pipelines, federated learning) now allows for scalable, secure handling of unstructured EHI elements. As these capabilities mature, the cost-benefit curve is shifting — with the marginal value of incorporating full EHI increasing while technical barriers are decreasing. However, realizing the full value of expanded EHI exchange requires more than just payload size. It demands robust, trustable, fine-grained access controls that honor complex legal, clinical, and patient-driven restrictions. CMS should actively support implementation of emerging standards such as Data Segmentation for Privacy (DS4P), OAuth-based consent frameworks, and UDAP Tiered Authorization, which collectively enable compliant, context-aware data sharing at scale. These tools are critical to ensure that sensitive data such as substance use disorder records or reproductive health information can be exchanged in a manner that is both legally permissible and technically secure.

Conclusion

Full EHI API access represents a necessary evolution in health information policy. While USCDI provides a valuable baseline for structured data exchange, it does not encompass the full scope of information required for effective care coordination, risk stratification, or patient-facing applications.

CMS should lead in establishing a phased, standards-aligned roadmap for expanding beyond USCDI, one that incorporates strong privacy protections, real-world operational use cases, and explicit technical safeguards. This includes support for emerging frameworks such as DS4P, OAuth-based consent management, and UDAP Tiered Authorization, which collectively enable compliant and patient-directed exchange at scale.

Equally important is a forward-looking approach to metadata architecture. As data ecosystems grow in complexity, the ability to assign rich, machine-readable metadata tags will be central not only to privacy enforcement, but to data provenance, quality tracking, version control, access governance, and AI-readiness. Metadata is the connective tissue that allows information to be understood, trusted, and reused across systems and over time. Its thoughtful and consistent application is the next frontier in data stewardship. CMS should take a leadership role in defining and incentivizing its adoption across health IT infrastructure.

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If done correctly, this expansion will transition the healthcare system from fragmented and provider-centric records to holistic, patient-centered data exchange — fueling innovation in care delivery, payment reform, and public health responsiveness.

TD 17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

Healthcare data sharing networks remain essential infrastructure for value-based care, public health surveillance, patient empowerment, and program integrity. Yet their long-term sustainability is uncertain without aligned incentives and infrastructure support. CMS and ONC should take complementary, non-overlapping roles that catalyze network effects, reduce participation barriers, and align payment models to create persistent demand. The goal is not to operate networks directly, but to ensure the conditions are in place for markets to function sustainably, consistently, and at scale.

The Nature of the Challenge

Health data networks face inherent market failures. Like roads or the internet backbone, they generate positive externalities that exceed what individual participants can monetize. Left solely to private actors, these networks often underperform due to fragmented incentives, lack of scale, or prohibitive onboarding costs. CMS and ONC are uniquely positioned to correct these failures. This is done not through perpetual funding, but by shaping markets, reducing friction, and aligning policy with operational realities.

CMS Should Shape Demand and Align Incentives

As the nation's largest healthcare payer, CMS holds unique leverage to drive sustained network participation by aligning its policies, payment models, and infrastructure investments. It should use this position to advance payer-to-payer exchange as a core national capability — essential not only for portability of coverage and continuity of care, but also for advancing real-time risk adjustment, beneficiary protection, and program oversight.

To create durable market demand, CMS should:

- **Embed data exchange expectations** into value-based care models and implement differential reimbursement to reward providers and plans that contribute complete, high-quality data.
- **Incentivize beneficiary participation** in longitudinal data sharing, particularly within Medicare Advantage and Medicaid managed care, using trusted applications and credentialing pathways.
- **Support the adoption of shared, standards-based infrastructure**, such as directory services, consent frameworks, and digital credentialing, by embedding them in program requirements and guidance. While CMS may not operate patient-matching utilities directly, it can strengthen match quality by advancing digital identity, demographic data standards, and interoperable metadata practices, enabling local and federated approaches to improve accuracy over time.
- **Ensure that CMS systems are designed to leverage common exchange infrastructure** for prior authorization, quality reporting, and coverage determination, thereby avoiding siloed builds and duplicative data flows.

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Finally, CMS should convene commercial payers, state Medicaid agencies, and network platforms to align with governance models, identity strategies, and incentive frameworks that support payer-to-payer exchange as the backbone of a connected, patient-centered data ecosystem.

ONC Should Lower Technical and Participation Barriers

ONC's responsibility is to reduce friction for participation, particularly among under-resourced and safety-net providers. This includes continuing to evolve FHIR, USCDI, and implementation guides with a focus on usability, backward compatibility, and implementation feasibility across diverse care settings. Certification programs should transition from measuring technical conformance to emphasizing real-world exchange performance.

ONC should expand technical assistance for rural and safety-net providers to support seamless integration with national health information networks. Innovation funding should be targeted to address specific scaling barriers - such as identity resolution, consent portability, and real-time data validation.

In parallel, ONC should actively coordinate with CMS to ensure its technical standards and certification criteria support payer-to-payer data exchange at scale. As CMS operationalizes its obligations under the Interoperability and Prior Authorization Final Rule (CMS-0057-F), ONC's infrastructure and standards must enable low-friction participation across Medicare FFS and Advantage, Medicaid managed care, and Qualified Health Plans, especially for longitudinal record transfer during transitions in coverage and care.

Shared Responsibilities and Sustained Implementation

The future of health data sharing depends not on federal operation of networks, but on a clear delineation of responsibilities and sustained collaboration between CMS and ONC. CMS should shape the demand environment by embedding data exchange into how healthcare is paid for and delivered. ONC should reduce technical complexity, expand participation, and ensure standards remain fit for purpose as policy and operational demands evolve.

Together, CMS and ONC must commit to a shared implementation approach that:

- **Sets clear expectations** for outcomes-based exchange performance
- Ensures that federal **programs and technical standards evolve in sync**, minimizing friction for implementers
- **Prioritizes alignment** across payer and provider networks, so data can follow the patient through transitions in both coverage and care
- **Supports the technical and policy foundations** for federated identity, consent enforcement, and metadata tagging — key enablers of longitudinal record assembly

This model of joint leadership is particularly urgent as CMS begins implementing the Interoperability and Prior Authorization Final Rule (CMS-0057-F), where payer-to-payer exchange is not only a compliance obligation but a strategic opportunity to build a more connected, patient-centered system. CMS can lead the charge in shaping demand and governance while ONC must ensure the infrastructure and standards are ready for scale.

If implemented with discipline and coordination, this approach will allow networks to evolve from grant-funded pilots to trusted, market-sustained utilities that support innovation and interoperability, reduce administrative burden, and deliver meaningful value across public and private sectors.

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